

ADMISSION NOTE - Patient synth_02 - Acute viral hepatitis (probable Hepatitis E)

CITYWIDE MULTISPECIALITY HOSPITAL, BANGALORE

ADMISSION NOTE

Patient Name: Arjun Singh

Age/Sex: 28/M

UHID: 12345678

Admission Date: 15/04/26

DIAGNOSIS:

1. Acute Febrile Illness with Jaundice ?Viral Hepatitis
2. Acute Gastritis
3. ?Acute Cholecystitis

CHIEF COMPLAINTS:

1. Fever with chills - 5 days
2. Yellowish discoloration of eyes and urine - 3 days
3. Nausea and vomiting - 3 days
4. Generalised weakness - 5 days

HISTORY OF PRESENT ILLNESS:

Patient, a 28-year-old male, was apparently well 5 days ago when he developed low-grade fever with chills. He took self-medication (TAB DOLO-650) with temporary relief. For the last 3 days, he has noted progressive yellowish discoloration of his sclera and passing dark-colored urine. This was associated with significant nausea and 4-5 episodes of non-bilious vomiting per day, decreased appetite, and a dull, aching pain in the right upper quadrant of his abdomen. He reports profound fatigue. There is a history of consuming food from a street vendor approximately 2 weeks prior. There is no history of cough, loose stools, or urinary symptoms. No history of recent travel, blood transfusions, IV drug use, or tattoos.

PAST HISTORY:

No known co-morbidities such as Diabetes Mellitus, Hypertension, Thyroid disease, or Tuberculosis. No previous surgical history.

PERSONAL HISTORY:

Occupation: Software Engineer. Non-smoker. Consumes alcohol occasionally, last intake over a month ago. Diet: Mixed.

GENERAL EXAMINATION:

Patient is conscious, alert, and oriented to time, place, and person. Obvious icterus is present in the sclera. Afebrile at presentation. PR: 92/min, regular. BP: 110/70 mmHg. RR: 18/min. SpO2: 99% on room air. No pallor, clubbing, cyanosis, or pedal edema.

SYSTEMIC EXAMINATION:

Abdomen: Soft, non-distended. Mild tenderness noted in the right hypochondrium on deep palpation. Liver is palpable 2cm below the right costal margin and is tender. Spleen is not palpable. Bowel sounds are present and normal.

Respiratory System: Bilateral air entry is equal and clear. No added sounds.

Cardiovascular System: S1, S2 heard. No murmurs.

Central Nervous System: GCS 15/15. No focal neurological deficits.

INVESTIGATIONS:

Blood samples sent for: Complete Blood Count, Liver Function Test, Renal Function Test, Serum Electrolytes, Viral Markers (HBsAg, Anti-HCV, Anti-HAV IgM, Anti-HEV IgM), Urine Routine & Microscopy. Reports are awaited.

PLAN:

Admit to ward. Start IV fluids. Symptomatic management for nausea and pain. Await blood reports for further management.

PROGRESS NOTE ? Day 1 - Patient synth_02 - Acute viral hepatitis (probable Hepatitis E)

CITYWIDE MULTISPECIALITY HOSPITAL, BANGALORE
PROGRESS NOTE

Patient Name: Arjun Singh, 28/M
Date: 16/04/26, 9:00 AM

SUBJECTIVE:

Patient reports feeling slightly better. Nausea has decreased, with only one episode of vomiting overnight. He still feels very weak and has a poor appetite. Continues to pass dark yellow urine. No new complaints.

OBJECTIVE:

General Condition: Conscious, oriented. Icterus ++. Afebrile.

Vitals: PR 88/min, BP 116/70 mmHg, RR 18/min, SpO2 98% RA.

Examination: Abdominal examination reveals persistent tenderness in the right hypochondrium. Liver remains palpable and tender.

INVESTIGATIONS UPDATE:

CBC: Hb 14.2 g/dL, TLC 7800/cumm, Platelets 1.9 lakhs/cumm

LFT: Total Bili 6.8 mg/dL (Direct 4.2), SGOT 1250 U/L, SGPT 1890 U/L, ALP 220 U/L

RFT: Creatinine 0.9 mg/dL

Viral Markers: HBsAg, Anti-HCV, Anti-HAV IgM - All Negative. Anti-HEV IgM - Awaited.

ASSESSMENT:

Acute Viral Hepatitis, likely Hepatitis E given history and negative serology for A, B, C.

PLAN:

1. Continue IV Fluids (DNS).
2. Continue INJ PAN-40 1-0-0 and INJ EMESET 1-1-1.
3. Add TAB URSODEOXYCHOLIC ACID 300mg 1-0-1 for cholestasis.
4. TAB DOLO-650 SOS for fever.
5. Encourage oral fluids and a low-fat, high-carbohydrate diet.
6. Monitor LFT tomorrow.
7. Await Anti-HEV IgM report.

PROGRESS NOTE ? Day 2 - Patient synth_02 - Acute viral hepatitis (probable Hepatitis E)

CITYWIDE MULTISPECIALITY HOSPITAL, BANGALORE
PROGRESS NOTE

Patient Name: Arjun Singh, 28/M
Date: 17/04/26, 9:30 AM

SUBJECTIVE:

Patient reports significant improvement. No episodes of vomiting in the last 24 hours. Appetite is better, and he has started taking soft diet. Feels less weak. Urine color is still dark but subjectively lighter.

OBJECTIVE:

General Condition: Looks comfortable. Icterus persists. Afebrile.

Vitals: PR 80/min, BP 120/80 mmHg, RR 16/min, SpO2 99% RA.

Examination: Abdominal tenderness has markedly reduced. Liver edge is still palpable but less tender.

INVESTIGATIONS UPDATE:

Repeat LFT: Total Bili 5.9 mg/dL, SGOT 850 U/L, SGPT 1300 U/L. (Showing a downward trend).

ASSESSMENT:

Resolving Acute Viral Hepatitis. Clinically and biochemically improving.

PLAN:

1. Patient is hemodynamically stable and tolerating orals well.
2. Stop IV fluids. IV cannula to be removed.
3. Convert injectable medications to oral route.
4. Plan for discharge today with advice and follow-up instructions.
5. To continue TAB PAN-40, TAB URSODEOXYCHOLIC ACID, and supportive vitamins.
6. Counsel patient regarding diet, rest, and alcohol avoidance.
7. Await final Anti-HEV IgM report and communicate telephonically.

LABORATORY REPORT - Patient synth_02 - Acute viral hepatitis (probable Hepatitis E)

CITYWIDE MULTISPECIALITY HOSPITAL, BANGALORE
LABORATORY REPORT

Patient Name: Arjun Singh, 28/M
UHID: 12345678
Sample Collection Date: 15/04/26, 11:00 AM
Report Date: 16/04/26, 8:00 AM

TEST NAME RESULT UNITS REFERENCE RANGE

****LIVER FUNCTION TEST****
Bilirubin - Total 6.8 mg/dL 0.2 - 1.2
Bilirubin - Direct 4.2 mg/dL 0.0 - 0.3
SGPT / ALT 1890 U/L < 40
SGOT / AST 1250 U/L < 40
Alkaline Phosphatase 220 U/L 44 - 147

****COMPLETE BLOOD COUNT****
Hemoglobin 14.2 g/dL 13.8 - 17.2
Total Leucocyte Count 7800 /cumm 4000 - 11000
Platelet Count 1.9 lakhs/cumm 1.5 - 4.5

****VIRAL MARKERS (SEROLOGY)****
HBsAg (Hepatitis B) Negative Negative
Anti-HCV (Hepatitis C) Negative Negative
Anti-HAV IgM (Hepatitis A)Negative Negative
Anti-HEV IgM (Hepatitis E)Awaited Negative

--- End of Report ---

MEDICATION RECORD - Patient synth_02 - Acute viral hepatitis (probable Hepatitis E)

CITYWIDE MULTISPECIALITY HOSPITAL, BANGALORE
MEDICATION RECORD

Patient Name: Arjun Singh, 28/M
UHID: 12345678

--- ADMISSION MEDICATIONS (From 15/04/26) ---

- 1. IV FLUID (DNS 500ml) - Route: IV - Frequency: 1-1-1 (8 hourly)
- 2. INJ PAN-40 (Pantoprazole 40mg) - Route: IV - Frequency: 1-0-0
- 3. INJ EMESSET (Ondansetron 4mg) - Route: IV - Frequency: 1-1-1
- 4. TAB URSODEOXYCHOLIC ACID 300mg - Route: Oral - Frequency: 1-0-1
- 5. TAB DOLO-650 (Paracetamol 650mg) - Route: Oral - Frequency: SOS for fever/pain

--- DISCHARGE MEDICATIONS (From 17/04/26) ---

- 1. TAB PANTOSEC (Pantoprazole 40mg) - Route: Oral - Frequency: 1-0-0 (before breakfast) - Duration: 1 week
- 2. TAB PERINORM (Metoclopramide 10mg) - Route: Oral - Frequency: SOS for nausea - Duration: As needed
- 3. TAB URSODEOXYCHOLIC ACID 300mg - Route: Oral - Frequency: 1-0-1 - Duration: 2 weeks
- 4. TAB BECOSULES Z (B-Complex with Vitamin C, Zinc) - Route: Oral - Frequency: 0-1-0 - Duration: 2 weeks
- 5. TAB DOLO-650 (Paracetamol 650mg) - Route: Oral - Frequency: SOS for pain/fever - Duration: As needed

DISCHARGE ADVICE - Patient synth_02 - Acute viral hepatitis (probable Hepatitis E)

CITYWIDE MULTISPECIALITY HOSPITAL, BANGALORE
DISCHARGE SUMMARY

Patient Name: Arjun Singh
Age/Sex: 28/M
UHID: 12345678
Date of Admission: 15/04/26
Date of Discharge: 17/04/26

- DIAGNOSIS:
- 1. Acute Viral Hepatitis (Probable Hepatitis E)
 - 2. Acute Gastritis
 - 3. Acute Cholecystitis (Resolved)

COURSE IN THE HOSPITAL:

Mr. Arjun Singh was admitted with a 5-day history of fever, followed by jaundice, nausea, and abdominal pain. Initial investigations showed significantly elevated liver enzymes (SGPT/SGOT >1000 U/L) with hyperbilirubinemia. Viral markers for Hepatitis A, B, and C were negative. He was managed conservatively with intravenous fluids and supportive medications. His clinical symptoms improved, liver enzymes showed a declining trend, and he began tolerating an oral diet well. Given his stable and improving condition, he is being discharged.

CONDITION AT DISCHARGE:

Clinically stable, afebrile, and hemodynamically stable. Patient is alert, oriented, and accepting oral diet and fluids. Icterus is resolving.

- ADVICE ON DISCHARGE:
- Medications:
- | No. | Medication Name | Dosage | Frequency | Duration |
|-----|--------------------------|--------|----------------------------|-----------|
| 1. | TAB PANTOSEC | 40 mg | 1-0-0 (Pre-breakfast) | 7 Days |
| 2. | TAB PERINORM | 10 mg | SOS for Nausea (Max 3/day) | As needed |
| 3. | TAB URSODEOXYCHOLIC ACID | 300 mg | 1-0-1 | 2 Weeks |
| 4. | TAB BECOSULES Z | 1 Cap | 0-1-0 | 2 Weeks |
| 5. | TAB DOLO-650 | 650 mg | SOS for Pain/Fever | As needed |

- Diet and Lifestyle:
- Take complete bed rest for at least 1-2 weeks. Avoid strenuous physical activity.
 - Diet: High carbohydrate, adequate protein, low-fat diet. Avoid oily, spicy, and outside food.
 - Drink plenty of fluids (2-3 litres/day) like water, juice, coconut water.
 - ABSOLUTE AVOIDANCE OF ALCOHOL for a minimum of 6 months.

- FOLLOW-UP INSTRUCTIONS:
- Review in the Gastroenterology OPD after 1 week with a repeat Liver Function Test (LFT) report.
 - Report to the emergency department in case of increased vomiting, severe abdominal pain, altered behaviour, or excessive sleepiness.

- PENDING RESULTS:
- The report for Anti-HEV IgM (Hepatitis E virus) is awaited. You will be contacted by the hospital with the result once it is available.